



Comfort Measures/End of Life Care

Assessment

Pediatric Pearls:

- Position parents/caregivers in close proximity to the patient
- Allow parents/caregivers to comfort patient and guide medically-appropriate care

Signs & Symptoms:

- Apnea/Agonal breathing
- Hypotension
- AMS
- Bradycardia

Differential:

- If there is any doubt about possible reversible causes, implement resuscitative measures and consult OLMC

Clinical Management Options

P	P	P	P	P	P		• Environment modification (dim lights, silence sirens/alarms) • Discontinue continuous monitoring (SpO2, cardiac monitor) if patient/family desires • Prioritize family and loved ones at bedside and allow them to play music or provide any other environmental comforts • Ice chips and/or sips of water if desired and able to tolerate secretions • Oxygen if desired or showing signs of air hunger
L	L	L	L	L	L		
1	2	3	4	5	6		• Establish vascular access only if patient/family prefer IV administration vs IM/IN
							• Fentanyl for pain and air hunger (1st line) • Midazolam for air hunger or anxiety • Droperidol for nausea/vomiting (1st line)

Consult Online Medical Control As Needed

Pearls:

- Implementation of comfort measures only/end of life care **REQUIRES** all of the following
 1. **A medical condition/injury where death is imminent**
 2. **A valid out of hospital DNR OR consultation with OLMC**
 3. **Consent and agreement from POA or recognized surrogate decision maker (not required if patient meets TOR criteria)**
- Terminal pain and anxiety can still occur in patients with limited consciousness and should be treated. Signs include tachycardia, hypertension, tachypnea, or grimace.
- Patients on chronic opioids may require medication doses that exceed the clinical guidelines. Contact OLMC for orders if higher doses are needed.
- Transportation to an emergency department while under hospice care should involve close communication with hospice nursing. Family or EMS personnel should attempt to contact the hospice agency if not already on scene. Allow hospice agency to communicate directly with family if there is a difference of opinion on transportation.
- May give initial doses of fentanyl and midazolam simultaneously
- Treatment should be patient-directed and given only to minimize pain and suffering, not to induce apnea or unconsciousness.